

9

Rehabilitation and Restorative Care



1. Discuss rehabilitation and restorative care

Define the following term:

rehabilitation

care that is given by specialists to help restore or improve function after an illness or injury.

restorative care

care given after rehabilitation to maintain a person's function, improve his quality of life and increase his independence.

1. Discuss rehabilitation and restorative care

Rehabilitation seeks to move the resident from

- Illness to health
- Disability to ability
- Dependence to independence

The goals of rehabilitation include the following:

- Help resident regain function or recover from illness
- Develop and promote a resident's independence
- Help a resident to feel in control of his life
- Help resident accept or adapt to the limitations of a disability

Transparency 9-1: Assisting with Rehabilitation and Restorative Care

- Be patient.
- Be positive and supportive.
- Focus on small tasks and small accomplishments.
- Recognize that setbacks occur.
- Be sensitive to the resident's needs.
- Encourage independence.
- Provide privacy.
- Involve residents in their care.

1. Discuss rehabilitation and restorative care

NAs should observe for and report the following signs and symptoms during rehabilitation and restorative care:

- Increase or decrease in abilities
- Change in attitude or motivation
- Change in general health
- Signs of depression or mood changes

2. Describe the importance of promoting independence and list ways exercise improves health

REMEMBER:

It is extremely important to encourage residents' independence while assisting with or performing all tasks. Studies show that the more active a person is, the better the mind and body work.

2. Describe the importance of promoting independence and list ways exercise improves health

Lack of mobility can cause these problems:

- Loss of self-esteem
- Depression
- Anxiety
- Boredom
- Pneumonia
- Urinary tract infection
- Skin breakdown and pressure injuries
- Constipation

2. Describe the importance of promoting independence and list ways exercise improves health

Lack of mobility can cause these problems (cont'd):

- Blood clots
- Dulling of senses
- Muscle atrophy
- Contractures
- Problems with independence and self-esteem

Transparency 9-2:Regular Ambulation and Exercise

Regular ambulation and exercise help improve the following:

- Quality and health of skin
- Circulation
- Strength
- Sleep and relaxation
- Mood
- Self-esteem
- Appetite
- Elimination
- Blood flow
- Oxygen level

3. Discuss ambulation and describe assistive devices and equipment

Define the following terms:

ambulation

walking.

ambulatory

capable of walking.

assistive/adaptive devices

special equipment that helps a person who is ill or disabled to perform activities of daily living.

3. Discuss ambulation and describe assistive devices and equipment

REMEMBER:

NAs should always check the care plan before helping a resident to ambulate and must keep the resident's limitations in mind.

3. Discuss ambulation and describe assistive devices and equipment

NAs should remember these guidelines for cane or walker use:

- Make sure cane or walker is in good condition.
- Make sure resident is wearing securely fastened, nonskid footwear.
- Resident should place cane on stronger side.
- Resident should place both hands on the walker, and walker should be placed no more than six inches in front of resident.
- Stay near resident on weaker side.
- Do not hang purses or clothing on walker.
- Report to nurse if cane or walker seems to be the wrong height.

<https://videos.hartmanonline.com/SkillsVideos/Ambulation/index.php?school=amtrain322>

Assisting a resident to ambulate

Equipment: gait belt, nonskid shoes for resident

1. Identify yourself by name. Identify the resident by name.
2. Wash your hands.
3. Explain procedure to resident. Speak clearly, slowly, and directly. Maintain face-to-face contact whenever possible.
4. Provide for resident's privacy with curtain, screen, or door.
5. Adjust bed to lowest position. Lock bed wheels. Assist the resident into a sitting position. Make sure his feet are flat on the floor. Adjust the bed height if needed.

Assisting a resident to ambulate

6. Before ambulating, put nonskid footwear on the resident and fasten securely.
7. Stand in front of and face the resident. Place your feet about shoulder-width apart.
8. Place gait belt around the resident's waist over his clothing (not on bare skin). Grasp belt securely on both sides with hands in the upward position.

Assisting a resident to ambulate

9. If resident is unable to stand without help, brace (support) the resident's lower extremities. This can be done by placing one or both of your knees against both of the resident's knees. Bend your knees. Keep your back straight.



Assisting a resident to ambulate

10. Hold the resident close to your center of gravity. Provide instructions to allow the resident to help with standing. Tell the resident to lean forward, push down on the bed with his hands, and stand on the count of three. When you start to count, begin to rock. On three, with hands still grasping the gait belt on both sides and moving upward, rock your weight onto your back foot. Slowly help the resident to stand.

Assisting a resident to ambulate

11. Walk slightly behind and to one side of the resident for the full distance, while holding onto the gait belt. If the resident has a weaker side, stand on the weaker side. Use the hand that is not holding the belt to offer support on the weaker side. Ask the resident to look forward, not down at the floor, during ambulation.



Assisting a resident to ambulate

12. After ambulation, remove the gait belt. Help the resident to the bed or chair. Check that the resident is in proper alignment
13. Leave bed in lowest position.
14. Place call light within resident's reach.
15. Wash your hands.
16. Report any changes in resident to nurse.
17. Document procedure using facility guidelines.

Assisting with ambulation for a resident using a cane, walker, or crutches

Equipment: gait belt, nonskid shoes for the resident, cane, walker, or crutches

1. Identify yourself by name. Identify the resident by name.
2. Wash your hands.
3. Explain procedure to resident. Speak clearly, slowly, and directly. Maintain face-to-face contact whenever possible.
4. Provide for resident's privacy with curtain, screen, or door.
5. Adjust bed to lowest position so that the feet are flat on the floor. Lock bed wheels.
6. Before ambulating, put nonskid footwear on resident and securely fasten.

Assisting with ambulation for a resident using a cane, walker, or crutches

7. Stand in front of and face resident. Place your feet about shoulder-width apart.
8. Place the gait belt around the resident's waist over his clothing (not on bare skin). Grasp the belt securely on both sides with hands in the upward position.
9. If resident is unable to stand without help, brace (support) resident's lower extremities (see previous procedure). Bend your knees. Keep your back straight. Help the resident to stand as described in the previous procedure.

Assisting with ambulation for a resident using a cane, walker, or crutches

10. Help as needed with ambulation.
 - a. **Cane:** Resident places cane about six inches, or a comfortable distance, in front of his stronger leg. He brings weaker leg even with cane. He then brings stronger leg forward slightly ahead of cane. Repeat.
 - b. **Walker:** Resident picks up or rolls the walker. He places it about six inches, or a comfortable distance, in front of him. All four feet or wheels of the walker should be on the ground before resident steps forward to the walker. The walker should not be moved again until the resident has moved both feet forward and is steady. The resident should never put his feet ahead of the walker.

Assisting with ambulation for a resident using a cane, walker, or crutches

- c. **Crutches:** Resident should be fitted for crutches and taught to use them correctly by a physical therapist or nurse. The resident may use crutches several different ways. It depends on what his weakness is. No matter how they are used, weight should be on the resident's hands and arms. Weight should not be on the underarm area.
- 11. Walk slightly behind and to one side of the resident for the full distance while holding the gait belt. If the resident has a weaker side, stand on the weaker side.
- 12. Watch for obstacles in the resident's path. Ask the resident to look forward, not down at the floor, during ambulation.

Assisting with ambulation for a resident using a cane, walker, or crutches

13. Encourage resident to rest if he is tired. When a person is tired, it increases the chance of a fall. Let the resident set the pace. Discuss how far he plans to go based on the care plan.
14. After ambulation, remove gait belt. Help resident to the bed or chair. Check that the resident is in proper alignment.
15. Leave bed in lowest position.
16. Place call light within resident's reach.
17. Wash your hands.
18. Report any changes in resident to nurse.
19. Document procedure using facility guidelines.

3. Discuss ambulation and describe assistive devices and equipment

REMEMBER:

There are many devices available to help people who are recovering from or adapting to a physical condition. NAs should be aware that residents may need time to adjust to using these devices. Residents using new aids to ambulation will likely be off-balance. NAs should stay close by and observe residents for signs of dizziness.

Proper body alignment aids recovery and prevents injury to muscles and joints.

Transparency 9-3 Proper Body Alignment:

- Observe principles of alignment.
- Keep body parts in natural positions.
- Prevent external rotation of hips.
- Change positions often, at least every two hours.
- Give back rubs as ordered.

5. Describe care guidelines for prosthetic devices

Define the following terms:

amputation

the surgical removal of some or all of a body part, usually a hand, arm, leg, or foot.

phantom sensation

warmth, itching, or tingling in a body part that has been amputated.

phantom limb pain

pain in a limb (or extremity) that has been amputated.

prosthesis

a device that replaces a body part that is missing or deformed because of an accident, injury, illness, or birth defect; used to improve a person's ability to function and/or to improve appearance.

5. Describe care guidelines for prosthetic devices

NAs should remember these guidelines for amputation and prosthetic devices:

- Be supportive.
- Help residents with their ADLs.
- Handle prostheses carefully and follow the care plan.
- Follow the nurse's or therapist's instructions in applying and removing the prosthesis. Follow manufacturer's care directions.
- Keep prosthesis and skin under it dry and clean.
- Apply stump sock if ordered.

5. Describe care guidelines for prosthetic devices

Guidelines for amputation and prosthetic devices (cont'd):

- Observe skin on stump and watch for signs of breakdown.
- Never try to fix a prosthesis.
- Do not show negative feelings about the stump during care.
- Treat phantom limb pain as real pain.
- If caring for an artificial eye, wash your hands before handling the eye. Wear gloves. Never clean or soak the eye in rubbing alcohol – it will crack and destroy it.
- Store artificial eye in water or saline. Make sure container is labeled with resident's name and room number.

6. Describe how to assist with range of motion exercises

Define the following terms:

range of motion (ROM)

exercises that put a joint through its full arc of motion.

abduction

moving a body part away from the midline of the body.

adduction

moving a body part toward the midline of the body.

dorsiflexion

bending backward.

rotation

turning a joint.

extension

straightening a body part.

6. Describe how to assist with range of motion exercises

Define the following terms:

flexion

bending a body part.

pronation

turning downward.

supination

turning upward.

opposition

touching the thumb to any other finger.

Transparency 9-4: Body Movements

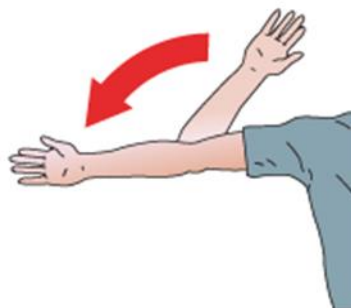
Abduction



Adduction



Extension



Flexion



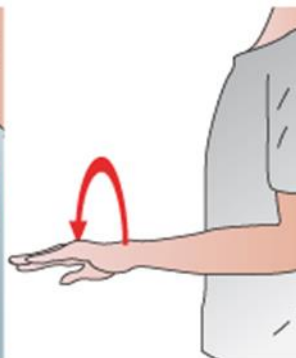
Dorsiflexion



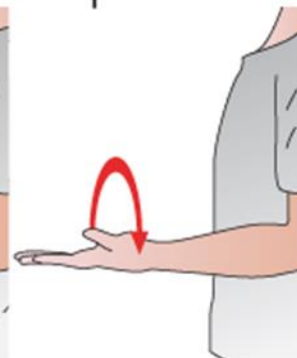
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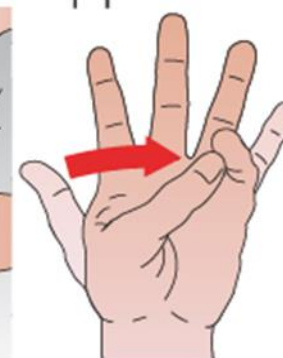
Pronation



Supination



Opposition



6. Describe how to assist with range of motion exercises

There are different types of ROM exercises:

- PROM: NA does all the work and resident does none.
- AROM: NA encourages, but resident does all the work.
- AAROM: NA assists and supports the resident in doing the work.

<https://videos.hartmanonline.com/SkillsVideos/PassiveRangeofMotion/index.php?school=amtrain322>

Assisting with passive range of motion exercises

1. Identify yourself by name. Identify the resident by name.
2. Wash your hands.
3. Explain procedure to resident. Speak clearly, slowly, and directly. Maintain face-to-face contact whenever possible.
4. Provide for resident's privacy with curtain, screen, or door.
5. Adjust bed to a safe level, usually waist high. Lock bed wheels.
6. Position the resident lying supine—flat on her back—on the bed. Use proper alignment. Ask the resident to let you know if she has any pain during the procedure.
7. While supporting the limbs, move all joints gently, slowly, and smoothly through the range of motion to the point of resistance. Repeat each exercise at least three times. Ask the resident if an exercise is causing pain. Stop if resident appears to be in pain or reports pain.

Assisting with passive range of motion exercises

8. **Shoulder:** Support the resident's arm at the elbow and wrist while performing ROM for the shoulder. Place one hand under the elbow and the other hand under the wrist. Raise the straightened arm from the side position upward toward head to ear level. Return arm down to side of the body (extension/flexion).



Assisting with passive range of motion exercises

Move straightened arm away from side of body to shoulder level. Return arm to side of body (abduction/adduction).



Assisting with passive range of motion exercises

9. **Elbow:** Hold the resident's wrist with one hand and the elbow with the other hand. Bend the elbow so that the hand touches the shoulder on that same side (flexion). Straighten the arm (extension)



Assisting with passive range of motion exercises

Exercise the forearm by moving it so the palm is facing downward (pronation) and then the palm is facing upward (supination).



Assisting with passive range of motion exercises

10. **Wrist:** Hold the wrist with one hand. Use the fingers of the other hand to help move the joint through the motions. Bend the hand down (flexion); bend the hand backward (dorsiflexion)



Assisting with passive range of motion exercises

Turn the hand in the direction of the thumb (radial flexion). Then turn the hand in the direction of the little finger (ulnar flexion).



Assisting with passive range of motion exercises

11. **Thumb:** Move the thumb away from the index finger (abduction). Move the thumb back next to the index finger (adduction).



Assisting with passive range of motion exercises

Touch each fingertip with the thumb (opposition).



Assisting with passive range of motion exercises

Bend thumb into the palm (flexion) and out to the side (extension).



Assisting with passive range of motion exercises

12. **Fingers:** Make the hand into a fist (flexion). Gently straighten out the fist (extension).



Assisting with passive range of motion exercises

Spread the fingers and the thumb far apart from each other (abduction). Bring the fingers back next to each other (adduction).



Assisting with passive range of motion exercises

13. **Hip:** Support the leg by placing one hand under the knee and one under the ankle. Straighten the leg and gently raise it upward. Move the leg away from the other leg (abduction). Move the leg toward the other leg (adduction).



Assisting with passive range of motion exercises

Gently turn the leg inward (internal rotation), then turn the leg outward (external rotation).



Assisting with passive range of motion exercises

14. **Knee:** Support the leg under the knee and under the ankle while performing ROM for the knee. Bend the knee to the point of resistance (flexion). Return leg to resident's normal position (extension).



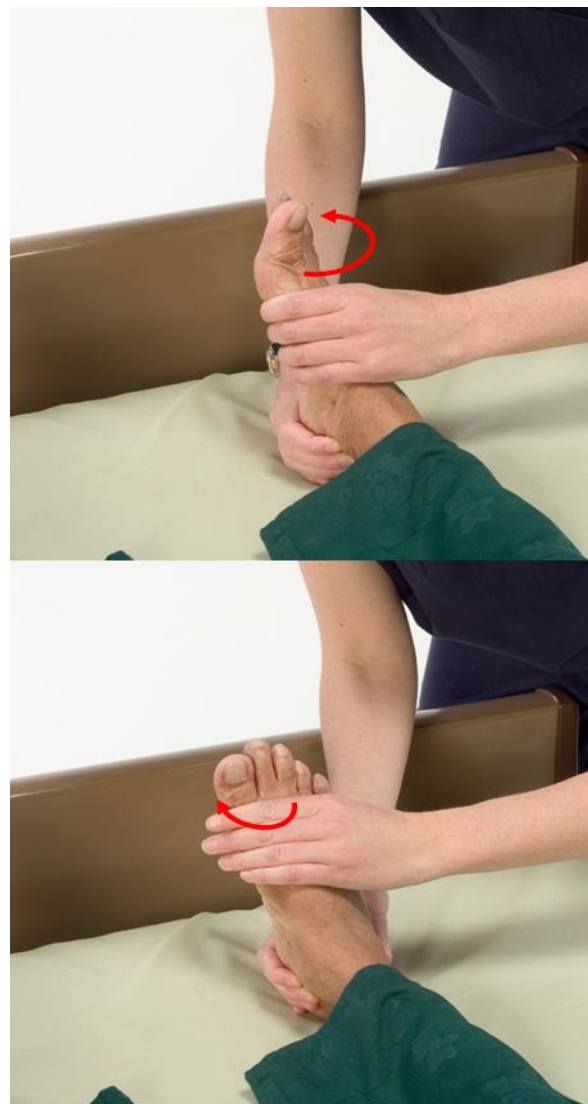
Assisting with passive range of motion exercises

15. **Ankle:** Support the foot and ankle close to the bed while performing ROM for the ankle. Push/pull foot up toward the head (dorsiflexion). Push/pull foot down, with the toes pointed down (plantar flexion).



Assisting with passive range of motion exercises

Turn the inside of the foot inward toward the body (supination). Bend the sole of the foot so that it faces away from the body (pronation).



Assisting with passive range of motion exercises

16. **Toes:** Curl and straighten the toes (flexion and extension).



Assisting with passive range of motion exercises

Gently spread the toes apart (abduction).



Assisting with passive range of motion exercises

17. Return resident to comfortable position. Return bed to lowest position.
18. Place call light within resident's reach.
19. Wash your hands.
20. Report any changes in resident to nurse.
21. Document procedure using facility guidelines. Note any decrease in range of motion or any pain experienced by the resident. Notify the nurse or the physical therapist if you find increased stiffness or physical resistance. Resistance may be a sign that a contracture is developing.

7. List guidelines for assisting with bladder and bowel retraining

When assisting with bladder or bowel retraining it is important that NAs follow these guidelines:

- Follow Standard Precautions. Wear gloves.
- Explain the schedule to the resident. Follow the schedule.
- Keep a record of resident's bladder and bowel habits. This will help you predict when a resident will need to eliminate.
- Offer a bedpan or a trip to the bathroom before long procedures.
- Encourage plenty of fluids.
- Encourage foods that are high in fiber.
- Answer call lights promptly.

7. List guidelines for assisting with bladder and bowel retraining

Guidelines for bowel and bladder retraining (cont'd):

- Provide privacy—both in the bed and in the bathroom.
- If resident has trouble urinating, try running water in the sink or suggest she lean forward slightly.
- Do not rush resident.
- Assist with careful perineal care.
- Discard wastes properly.
- Discard clothing protectors and incontinence briefs properly.

7. List guidelines for assisting with bladder and bowel retraining

Guidelines for bowel and bladder retraining (cont'd):

- If your facility uses washable bed protectors or briefs, follow Standard Precautions when handling these items.
- Keep an accurate record of urination and bowel movements, including episodes of incontinence.
- Praise successes and attempts to control bladder or bowels. However, do not talk to residents as if they were children.
- Never show frustration or anger.
- Be positive and patient. Praise and encouragement are essential. Report changes in the skin.

7. List guidelines for assisting with bladder and bowel retraining

REMEMBER:

It is very important that NAs keep a positive attitude when assisting residents who are going through retraining. Imagining what it would feel like to be unable to control elimination can help create empathy for the residents.



Hartman